



PAPUA NEW GUINEA NATIONAL DEPARTMENT OF HEALTH

The National Department Of Health Regulatory Bodies Nursing Council & The Medical Board Online Workforce System

Full-Scope Platform User Guide

Plain-language guide for staff, registrars, administrators, finance, data quality, and document repository users.

Generated: 07 May 2026

OpenKM Timeline Implementation Check

Implementation Audit Against OpenKM Timeline

Timeline area	Status	Where implemented
Phase 1 Repository Foundation	Completed and integrated	apps.documents models, admin, URLs, staff screens, bootstrap_document_repository, 24 folders.
Phase 2 OCR/Search/Duplicate	Completed and integrated	DocumentVersion extracted text, OCR linkage, reference extraction, repository search, checksum duplicate grouping.
Phase 3 Workflow/Staff Review	Completed and integrated	Nursing Council workflow config, checklist review, linked repository evidence on application detail, inbox/access requests.
Phase 4 Governance/Security	Completed and integrated	System Admin-only admin, office scope separation, finance separation, public-safe register, audit events.
Phase 5 Documentation/Training	Completed	Updated manuals, cleansing guide, OpenKM guide, project timeline, and generated PDF handover guides.

Plain-language conclusion: the OpenKM-style functions are integrated into the platform. The remaining work is operational adoption: staff must scan paper records, upload evidence, tag metadata, link documents to applications or practitioners, and keep data-quality review active.

Plain Language Staff Guide

How Staff Use The Platform In Plain Language

Staff group	What to do
System Admin	Set up users, repository folders, access rules, bootstraps, backups, and production settings.
Registrar	Review applications, open linked evidence, verify checklist/payment/competency, approve or reject, and generate reports.
Reviewer	Request operational access first. After approval, help review assigned records and evidence only within scope.
Data Quality Officer	Review missing data, duplicates, source rows, and repository evidence before reports are published.
Finance Officer	Use separated Nursing Council and Medical Board financial forecast pages. Do not edit records or mix office figures.
Professional / Applicant	Use own portal, own applications, own receipts, and public-safe forms. Cannot view other people's private records.

For documents: open Document Repository, upload the scanned evidence, choose the right office scope, add metadata, link it to the application or practitioner, run OCR if needed, and use the audit trail to prove who viewed, uploaded, downloaded, or changed the record.

Main User Manual

Full Platform User Guidelines and Manual

Project: The National Department Of Health Regulatory Bodies Nursing Council & The Medical Board Online Workforce System

Date: 07 May 2026

Audience: Registrars, reviewers, finance officers, data-quality officers, System Admin, professional users, graduands, training institutions, and authorised NDOH staff.

1. Purpose Of This Guide

This guide explains how staff and users should use the current platform in plain language.

The platform is no longer just a set of forms or dashboards. It is now a regulatory operations system for:

- Online registration and licence workflows.
- Nursing Council and Medical Board protected workspaces.
- Practitioner profiles and workforce records.
- Applications, renewals, provisional records, full registration, temporary licences, and overseas pathways.
- Receipts, financial forecast reports, and office-separated financial tracking.
- Document repository, scanned records, metadata, versions, OCR/search text, and audit trails.
- Missing-data review, duplicate review, import cleansing, and production-readiness checks.
- Staff inbox, chat, notifications, operational access requests, and AI Staff Assistant guidance.
- Management reports, monthly analytics, yearly analytics, Word briefs, Excel workbooks, and PDF reports.

The main goal is simple: move paper and spreadsheet records into a controlled electronic registry without mixing offices, exposing private data, or trusting unverified imported rows.

2. The Most Important Rule

Nursing Council and Medical Board data must remain separate.

Nursing Council users must not view private Medical Board data unless they are explicitly authorised. Medical Board users must not view private Nursing Council data unless they are explicitly authorised. This is enforced by backend role checks, not just by hiding buttons.

The same principle applies to finance:

- Nursing Council receipts and forecasts must be viewed separately.
- Medical Board receipts and forecasts must be viewed separately.
- Combined or mixed totals should only be used where management has specifically requested a combined summary and the report clearly labels the source.

3. Current Platform Areas

Area	What it is used for	Who normally uses it
Public home and login pages	Public entry, sign in, password reset, and basic system access.	All users.
Overall Dashboard	High-level workforce summary and reference count explanation.	System Admin, registrars, approved staff.

Area	What it is used for	Who normally uses it
Nursing Council Portal	Nursing Council live statistics, ATP/current-year records, workflows, reports, and cleansing views.	Nursing Registrar, approved Nursing staff, System Admin.
Medical Board Portal	Medical Board-specific records, statistics, and workflow views.	Medical Board Registrar, approved Medical Board staff, System Admin.
Workforce Flow	Visual explanation of registration and workforce data movement.	Registrars, finance officer, reviewers, approved staff.
Financial Forecast	Manual receipts, spreadsheet receipts, monthly/yearly totals, forecast charts, and exports.	Registrar, finance officer, System Admin.
Records Hub	Structured table review and controlled record management.	System Admin, registrars, approved data/review staff.
Document Repository	OpenKM-style scanned records, versions, metadata, OCR/search, and audit logs.	Authorised staff only.
Production Readiness Dashboard	Remaining launch risks, data problems, security checks, and operational readiness indicators.	System Admin, registrars, project leads.
Staff Inbox & Chat	Internal messages, notifications, access requests, document/application follow-up.	Staff users.
AI Staff Assistant	Staff-only guidance for reports, workflow, imports, and data-quality questions.	Authorised staff only.

4. Current Live Statistics Snapshot

These values are from the current live platform state on 07 May 2026. They will change as staff import, cleanse, approve, and update records.

Measure	Current value	How to understand it
Registered nurses	13,493	Live Nursing Council nurse records currently held in the registry table.
Midwives	2,112	Live midwife records currently held in the registry table.
Nurse aides	800	Live nurse aide records currently held in the registry table.
Graduands	7,624	Live graduand or health student records currently held in the system.
Community Health Workers	11,562	CHW records currently available in the wider platform data.
Medical doctors	0	Medical doctor data is still being updated or has not yet been loaded into the live medical doctor table.
PNG Nursing schools	20	Clean operational count for Nursing Council schools, separated from raw imported institution names.
Raw institution rows	916	Historical/imported institution references before final classification and cleansing.
Cleaned workplace references	4,926	Workplace/facility names cleaned from imported records. These are not all confirmed master facilities.
Raw workplace/address rows	7,439	Historical/raw workplace address references, including repeated and incomplete entries.

Measure	Current value	How to understand it
Missing-data review items	98,055	Records or rows with fields needing review, such as missing dates, province, gender, workplace, or institution.
Duplicate-review practitioners	487	Practitioner candidates requiring manual duplicate review before merging or confirming.
Nursing finance combined receipts	PGK 1,420,842.20	Nursing Council manual and spreadsheet receipt data combined for the current finance scope.

Important: live person counts are not the same as imported row counts. One person can appear in many imported rows because of renewals, receipts, workforce listings, or historical spreadsheet entries.

5. Login, Password Reset, And Account Security

Use the correct account for your role. Do not share accounts.

Login steps:

- Open the platform in the browser.
- Select the correct login page.
- Enter your username and password.
- Use Forgotten Password if you cannot remember your password.
- After login, the system sends you to the dashboard allowed for your role.
- If your dashboard looks empty or locked, check your Profile Overview and submit an operational access request if required.

Security rules:

- Change all testing passwords before production launch.
- Use strong passwords.
- Do not use another officer's account to approve, export, or edit records.
- System Admin and registrar accounts should use MFA before production.
- Logout when leaving the computer.
- If a password reset email is used, open the latest reset link only.
- If a reset link fails after login in another tab, reload the page and request a fresh reset link.

6. Profile Overview And Profile Settings

The Profile Overview explains the role of the logged-in user.

It should tell the user:

- What their role is.
- What they can access.
- What they cannot access.
- Whether they need approval before using operational tools.
- Whether they can create, read, update, or delete records.
- Whether they can approve applications or only review them.

Profile settings are kept out of the middle of the profile page. Use the Profile Settings button near the signed-in user and logout section in the top bar to open or update settings.

If your role needs higher access, do not ask someone for their password. Submit an operational access request from your profile or staff workflow screen.

7. Operational Access Request Workflow

Some users can log in but cannot use sensitive tools immediately. This is intentional.

Workflow:

```
Restricted staff user logs in
-> opens My Profile
-> reads Profile Overview
-> submits operational access request
-> request appears in Staff Inbox & Chat for registrar/System Admin
-> registrar or System Admin reviews the request
-> request is approved or rejected
-> user access is updated
-> user logs out and back in if menu access needs refreshing
```

Nursing Council requests go to the Nursing Council Registrar and System Admin. Medical Board requests go to the Medical Board Registrar and System Admin.

The request is not approved automatically. This prevents reviewers, finance officers, viewers, or other staff from using high-risk tools without authority.

8. Role Access Guide

Role	Main purpose	Can access	Cannot access
System Admin	Technical administration and production control.	User setup, roles, backend admin, configuration, document repository setup, bootstraps, data-quality tools, production readiness, documentation, and system checks.	Should not be used for routine registrar decisions unless officially authorised to act in that capacity.
Nursing Council Registrar	Official Nursing Council regulatory decision maker.	Nursing Council dashboard, applications, workflow tools, approvals, rejections, reports, data-quality views, Nursing finance, staff access requests, and Nursing documents.	Medical Board private records unless specifically authorised.
Medical Board Registrar	Official Medical Board regulatory decision maker.	Medical Board dashboard, Medical Board workflows, Medical Board reports, Medical Board finance, and Medical Board staff access requests.	Nursing Council private records unless specifically authorised.
Nursing Reviewer	Supports Nursing Council review work.	Assigned review tasks after approval, selected document checks, selected records, and access request workflow.	Final registrar decisions, unrestricted Nursing Operations before approval, admin backend, Medical Board private data.
Medical Reviewer	Supports Medical Board review work.	Assigned Medical Board review tasks after approval.	Final registrar decisions, unrestricted Medical Board Operations before approval, admin backend, Nursing Council private data.
Data Quality Officer	Cleans and checks data.	Missing-data review, duplicate review, import issue review, source row checking, cleansing notes, and selected records.	Registrar approvals, finance approvals, admin backend, and private records outside assigned scope.

Role	Main purpose	Can access	Cannot access
Finance Officer	Reviews financial information without editing registry records.	Workforce Flow and separated Financial Forecast pages for Nursing Council and Medical Board.	CRUD functions, practitioner editing, application approval, admin backend, mixed uncontrolled finance views.
Viewer	Limited read-only user.	Public-safe or assigned views only.	CRUD, imports, approvals, exports where restricted, and private records.
Nurse	Professional self-service user.	Own profile, own applications, own receipts, own licence status, own documents.	Other people's records, staff dashboards, finance reports, admin backend.
Midwife	Professional self-service user.	Own profile, own applications, own receipts, own licence status, own documents.	Other people's records and staff-only tools.
Nurse Aide	Professional self-service user.	Own profile, own application/receipt status, own supporting documents.	Other people's records and registrar tools.
Graduand	Student or graduate pathway user.	Own graduand profile, pathway guidance, own applications, own receipts, supporting documents.	Staff dashboards, other graduands' records, registrar tools.
CHW	CHW self-service user.	Own CHW profile and available CHW/Medical Board information.	Nursing Council private records and staff tools.
Doctor	Doctor self-service user.	Own Medical Board profile and available doctor information.	Nursing Council private records and staff tools.
Public applicant	Public application or public register user.	Public-safe pages, own submitted information, public register search.	Private practitioner details, staff tools, admin backend.

9. System Admin Guide

The System Admin is responsible for the technical health of the platform.

System Admin tasks:

- Manage users and roles.
- Restrict backend administration access to System Admin only.
- Set up or update document repository folders.
- Run setup and bootstrap commands.
- Review production readiness.
- Review security configuration.
- Review audit logs.
- Confirm backups and restore readiness.
- Support registrars with approved access changes.
- Support documentation and deployment preparation.

System Admin must not casually override regulatory decisions. Registrar decisions need proper authority, status history, and audit trails.

10. Nursing Council Registrar Guide

Open the Nursing Council portal from the dashboard menu.

Registrar daily tasks:

- Review live Nursing Council statistics.
- Review ATP/current-year records and recent licence records.
- Review pending, approved, rejected, and missing-information applications.
- Check document checklists.
- Check payment status.
- Check competency evidence where required.
- Check duplicate-review and missing-data issues before reports are generated.
- Review access requests from Nursing Council reviewers or staff.
- Approve, reject, or request more information using the official workflow.
- Generate monthly, yearly, finance, and management reports only after data checks.

Registrar approval rules:

- Do not approve if required documents are missing.
- Do not approve if payment is required but not verified or waived.
- Do not approve full/provisional-to-full registration if competency is required but incomplete.
- Do not approve duplicate practitioner numbers without data-quality review.
- Do not use Nursing Council regulations for Medical Board records.
- Do not make silent decisions outside the workflow.

11. Medical Board Registrar Guide

Medical Board work must remain separate from Nursing Council work.

Medical Board registrar tasks:

- Review Medical Board records and workflows.
- Review Medical Board finance separately from Nursing Council finance.
- Review CHW and doctor records when available.
- Review Medical Board staff access requests.
- Generate Medical Board reports using Medical Board scope only.
- Apply Medical Board rules, not Nursing Council rules.

If a Medical Board dashboard link redirects incorrectly, report it to System Admin so URL routing can be corrected without using another role's account.

12. Reviewer Guide

Reviewers help prepare work for the registrar.

Before operational approval:

- Reviewer can log in.
- Reviewer can see their Profile Overview.
- Reviewer can request operational access.
- Sensitive Nursing Council or Medical Board Operations remain locked.

After operational approval:

- Reviewer can review assigned records.

- Reviewer can help check documents and missing data.
- Reviewer can prepare notes for registrar review.
- Reviewer can use only the tools granted to that role.

Reviewers cannot:

- Make final registrar approval decisions unless explicitly authorised by role and business authority.
- Use admin backend.
- View another office's private data.
- Use another staff account.

13. Finance Officer Guide

The Finance Officer has restricted access.

Allowed pages:

- Workforce Flow.
- Nursing Council Financial Forecast page.
- Medical Board Financial Forecast page.

Finance pages must be viewed separately:

- Nursing Council finance: `/dashboard/reports/financial/?office=nursing`
- Medical Board finance: `/dashboard/reports/financial/?office=medical`

Finance Officer can:

- View manual receipts.
- View spreadsheet-imported receipts.
- View monthly and yearly totals.
- View charts and forecast outlook.
- Export finance reports where permitted.
- Check date-quality warnings and suspicious receipt rows.

Finance Officer cannot:

- Create, edit, or delete practitioner records.
- Approve or reject applications.
- Use broad CRUD tools.
- Access backend administration.
- Mix Nursing Council and Medical Board financial information in one uncontrolled report.

How to read finance figures:

- Manual Receipts come from live receipt records entered through the system.
- Spreadsheet Receipts come from imported workbook payment rows.
- Combined Total is manual receipts plus spreadsheet receipts for the selected office scope.
- Current Month Total should only count receipts in the current month.
- Current Year Total should only count receipts in the current year.
- Forecast Month should not show future years such as 2050 unless the source data contains an incorrect future date.

If a receipt date shows an impossible year, report it as a data-quality issue before publishing the report.

14. Data Quality Officer Guide

Data-quality work protects the platform from producing wrong management figures.

Common issues to check:

- Same name appearing more than once.
- Same licence number appearing more than once.
- Same registration number appearing more than once.
- Same practitioner number appearing more than once.
- Missing first name or surname.
- Missing gender.
- Missing date of birth.
- Missing province.
- Missing institution.
- Missing workplace or facility.
- Missing licence issue date or expiry date.
- Missing payment date or amount.
- Incorrect future dates, including accidental 2050 dates.
- Overseas or raw imported institution names mixed with PNG nursing schools.

Data-quality rules:

- Do not delete historical rows just because they are messy.
- Do not overwrite good live records with blank spreadsheet values.
- Do not create a new practitioner when a registration number or practitioner number already matches an existing person.
- Keep the source file, sheet name, row number, and correction note.
- Resolve duplicates only when evidence confirms they are the same person.

15. Production Readiness Dashboard

The Production Readiness Dashboard makes remaining launch risks visible.

It should be used to monitor:

- Missing-data backlog.
- Duplicate-review backlog.
- Future-date and suspicious-date issues.
- Institution and facility classification issues.
- Finance date and receipt quality issues.
- Backup and restore readiness.
- Security and role access checks.
- Documentation and training status.
- Open launch actions before production handover.

Use this dashboard during weekly readiness meetings so data problems are managed openly instead of hidden in command output.

16. Overall Dashboard Guide

The Overall Dashboard provides a high-level picture of the platform.

It shows:

- Medical Doctors.
- Nursing Professionals.
- Midwives.
- Nurse Aides.
- Graduands.
- Community Health Workers.
- Facilities or workplace references.
- PNG Nursing Schools.
- Training institution breakdown.
- Facility/workplace breakdown.

Important explanation:

- PNG Nursing Schools is the cleaned operational Nursing Council school count.
- Raw institution rows include legacy, overseas, duplicate, and incomplete import references.
- Facility/workplace counts may include imported workplace text and are not always confirmed master facility records.
- Use the breakdown tables before quoting institution or facility totals in management documents.

17. Nursing Council Dashboard Guide

The Nursing Council dashboard is the main operational screen for the Nursing Council Registrar and approved staff.

It should show:

- Current Nursing Council live statistics.
- ATP/current-year information.
- Gender distribution.
- Provincial distribution.
- Workplace and sector distribution.
- PNG nursing school and institution breakdown.
- Recent full registration and licence records.
- Provisional licence tracking and expiry view.
- Full registration and practising licence by year.
- Nursing Council Operations tools.
- Missing-data and duplicate-review links.
- Report generation buttons.

If a chart is blank:

- Check whether the source table has current records.

- Check whether the data is scoped to Nursing Council only.
- Check whether province, year, workplace, or licence dates are missing.
- Run data-quality review before assuming the count is zero.

18. Workforce Flow Guide

The Workforce Flow page explains how records move through the system.

Plain workflow:

```
Applicant or staff submits data
-> documents and receipts are attached
-> system checks required fields
-> reviewer checks records
-> registrar makes official decision
-> licence or registry record is updated
-> dashboards and reports update
```

Finance Officer can view Workforce Flow, but cannot use it to edit practitioner records.

If Workforce Flow redirects to the wrong dashboard, report the issue. Do not use another user's account to bypass it.

19. Records Hub Guide

The Records Hub is used to view and manage structured data tables where authorised.

Large tables such as Applications and Receipts may load slowly because they contain many records. Use filters and search before opening large edit forms.

Best practice:

- Search first.
- Filter by office, status, year, category, or source where possible.
- Open one record at a time.
- Do not make unnecessary edits.
- Add correction notes where data is changed.
- Do not use Records Hub for registrar decisions when an application workflow exists.

20. Nursing Council Forms And Pathways

The Nursing Council forms page is now a pathway configuration screen, not just a PDF download page.

Configured pathway groups include:

- PNG Graduate Nurse Provisional Licence.
- PNG Graduate Midwife Full Registration.
- PNG Full Registration.
- PNG Licence Renewal.
- Provisional to Full.
- Overseas Provisional.
- Overseas Full.
- Temporary Overseas Licence.
- Child Health Specialist.
- Double Major Registration.
- Employer Verification.

- Deceased Notification.

The intended workflow is:

```
Application pathway
-> required form
-> required checklist
-> required documents
-> required fee rule
-> validation rules
-> staff review
-> registrar decision
-> licence/register update
-> dashboard/report update
```

Users must follow the correct pathway. For example, a midwifery graduate should not be processed as an ordinary renewal if the correct midwifery graduate pathway applies.

21. Application Review And Approval

Every official application should follow a controlled lifecycle.

Standard lifecycle:

```
Draft
-> Submitted
-> Payment Pending or Intake Screening
-> Missing Information or Data Quality Review if needed
-> Technical Review
-> Registrar Review
-> Approved or Rejected
-> Licence Issued or Register Updated
```

Rules:

- No silent approval.
- No undocumented decision.
- Every status change should have history.
- Every sensitive action should have audit logging.
- Required documents must be checked before registrar approval.
- Required payment must be verified or waived.
- Required declarations must be accepted.
- Required competency must be completed where applicable.
- Duplicate concerns must be resolved before publishing final counts.

22. Document Repository Guide

Open /documents/search/ if authorised.

The document repository is the OpenKM-style official records area inside the platform.

It supports:

- Office-scoped folders.
- Nursing Council documents.
- Medical Board documents.
- General Registry documents.
- Scanned forms.
- Receipts.
- Qualifications.

- Letters.
- Competency documents.
- Document metadata.
- Document versions.
- Extracted OCR/search text.
- File checksum duplicate detection.
- Official record flags.
- Retention years.
- Access policies.
- Audit events.

How to use it:

- Open the document repository.
- Select or confirm the correct office scope.
- Upload or locate the document.
- Add title, description, document type, source, and metadata.
- Link the document to the related practitioner, application, receipt, or workflow if available.
- Add a new version instead of replacing evidence silently.
- Search by name, reference number, receipt number, practitioner number, registration number, source text, or extracted OCR text.
- Review duplicate checksum groups if the same file has been uploaded more than once.

Do not store Nursing Council documents under Medical Board folders. Do not store Medical Board documents under Nursing Council folders.

23. AI Staff Assistant Guide

The AI Staff Assistant is for authorised staff guidance.

It can help with:

- Explaining dashboard numbers.
- Choosing which report to generate.
- Explaining missing-data issues.
- Explaining import and cleansing steps.
- Suggesting what workflow action to check next.
- Helping staff understand finance, records, or document repository screens.

It cannot:

- Approve applications.
- Reject applications.
- Verify payments.
- Override role permissions.
- Replace source documents.
- Make legal, disciplinary, or final regulatory decisions.

- Change live records by itself.

Current AI setup:

- The platform defaults to a safe local rule-based assistant.
- A free local GPT-style model can be connected through Ollama if installed by ICT or System Admin.
- OpenAI is not enabled by default because it is not free and requires external API billing.
- Imported data cleansing can use offline rules first, and optional local model suggestions only when configured.

If the floating AI button blocks another button, move it before clicking the page control. If the popup opens as a full page, use the full page safely and report the popup issue for UI correction.

24. Import And Cleansing Guide

The platform should not trust imported spreadsheets automatically.

Correct import rule:

```
Raw file
-> staging
-> validation
-> cleansing
-> duplicate review
-> staff review
-> registrar or authorised approval
-> live registry update
-> dashboard/report update
```

Import checks:

- Names.
- Gender.
- Date of birth.
- Registration number.
- Practitioner number.
- Licence number.
- ATP number.
- Qualification.
- Institution.
- Facility or workplace.
- Province.
- Employment status.
- Receipt number.
- Payment amount.
- Payment date.
- Source sheet.
- Source row.
- Import batch.

Good import practice:

- Preview before committing.
- Do not silently drop invalid rows.

- Do not insert unknown people directly into the live register.
- Do not overwrite verified live data with blank import values.
- Keep source file and row references.
- Send uncertain matches to duplicate review.
- Send incomplete rows to missing-data review.

25. Institution And Facility Breakdown Guide

Institution and facility figures must be explained carefully.

PNG Nursing Schools:

- This is the clean Nursing Council operational school count.
- Current clean count: 20.
- Government and non-government schools should be separated where known.
- Unclassified or review-required schools should remain flagged until confirmed.

Raw Training Institution Rows:

- These come from imported records.
- They may include overseas institutions, old spelling variations, duplicates, or non-nursing entries.
- Do not present raw imported institution rows as the official number of PNG nursing schools.

Facility and Workplace Rows:

- These may come from imported workplace text, address fields, CHW records, overseas records, and legacy spreadsheets.
- They must be classified into confirmed health facilities, workplaces, government, church, private, overseas, unknown, or review-required groups.
- Use breakdown tables when explaining the difference between master facilities and raw workplace references.

26. Financial Forecast And Reports Guide

Open the correct office-specific finance page:

- Nursing Council finance: </dashboard/reports/financial/?office=nursing>
- Medical Board finance: </dashboard/reports/financial/?office=medical>

Finance screens should show:

- Manual receipt total.
- Spreadsheet receipt total.
- Combined total.
- Current month total.
- Current year total.
- Completed manual receipts.
- Imported spreadsheet rows.
- Recent receipt transactions.
- Monthly trend chart.
- Yearly trend chart.

- Category breakdown.
- Forecast outlook.
- Date-quality warnings.

Export formats:

- Excel.
- PDF.
- Word.

Export rules:

- Confirm the selected office scope before exporting.
- Each export should include timestamp and exporting user.
- Each export should include NDOH logo or official heading.
- Do not publish reports with impossible future dates until reviewed.
- Explain manual receipts separately from spreadsheet receipts.
- Explain payment rows separately from practitioner counts.

27. Reports And Management Briefs

Reports should be generated after data-quality checks, not before.

Common reports:

- Monthly analytics.
- Yearly analytics.
- Nursing Council live statistics.
- Financial forecast.
- Production readiness.
- Minister brief.
- Registrar brief.
- Secretary brief.
- Documentation index.
- Government launch package.
- Presentation pack.

Every report should clearly state:

- Date generated.
- User who generated it if confidential.
- Office scope.
- Source table or source sheet.
- Whether figures are live people counts or imported rows.
- Whether duplicate and missing-data issues remain.
- Latest import date.
- Latest live record update date.

28. Presentation And Handover Documents

Generated presentation documents are stored in docs/presentation/.

Current presentation outputs include:

- NDOH_Regulatory_Platform_Presentation_Pack_20260507.pdf
- NDOH_Regulatory_Platform_Presentation_Brief_20260507.docx
- NDOH_Regulatory_Platform_Documentation_Index_20260507.pdf
- Screenshot assets for major user interfaces.
- Architecture, workflow, data-governance, and role/privacy diagrams.

Use these documents for management presentation, training, and handover. Regenerate them after major UI, role, or statistics changes.

29. Public Register And Public-Safe Access

Public register search must return safe fields only.

Allowed public fields:

- Full name.
- Registration number.
- Practitioner number if policy allows.
- Professional category.
- Licence status.
- Licence expiry date.
- Eligible-to-practise indicator.

Never expose publicly:

- Date of birth.
- Phone.
- Email.
- Home address.
- Passport.
- Police clearance.
- Medical report.
- Academic transcript.
- Receipt document.
- Private disciplinary notes.
- Internal review notes.

The public register is an output from the master register. It is not the source of truth.

30. Professional, Graduand, CHW, And Doctor User Guide

Self-service users can:

- View their own profile.
- View their own application status.

- View their own receipts.
- Submit applicable forms.
- Upload required supporting documents where enabled.
- Use Forgotten Password.
- Read fee and guideline pages.

Self-service users cannot:

- View another person's private record.
- Access staff dashboards.
- Access finance reports.
- Access document repository.
- Access Records Hub.
- Use registrar tools.
- Access backend administration.

Quick links such as handbooks, registration guidelines, and academic calendar should open the correct information page or document. If a quick link redirects to the wrong page, report it for correction.

31. Staff Inbox, Chat, And Notifications

Staff Inbox & Chat is used for internal operational communication.

It supports:

- Access requests.
- Enquiries.
- Review follow-up.
- Document review notices.
- Approval or rejection messages.
- Staff notifications.

Use staff chat for system workflow communication, not personal account sharing or unofficial approvals.

32. Daily, Weekly, And Monthly Work Routine

Daily:

- Check inbox and notifications.
- Check pending applications.
- Check missing-data or duplicate alerts.
- Check urgent finance or receipt issues.
- Check access requests if registrar or System Admin.

Weekly:

- Review production readiness dashboard.
- Review missing-data backlog.
- Review duplicate-review backlog.
- Review future-date and suspicious receipt issues.

- Review document repository records missing metadata.
- Review staff access permissions.

Monthly:

- Run data-quality checks.
- Generate monthly analytics.
- Generate financial forecast report.
- Review office-separated receipts.
- Prepare registrar or management brief.
- Check backup and restore evidence.
- Confirm source dates and latest import batches.

33. Privacy And Security Rules

Always follow these rules:

- Use your own account only.
- Keep Nursing Council and Medical Board data separated.
- Do not export confidential information unless required for official work.
- Do not send private data through personal email or messaging.
- Do not approve applications without required evidence.
- Do not bypass workflow because a record appears urgent.
- Do not use screenshots of private data in public presentations.
- Do not publish raw imported rows as official counts.
- Report suspicious dates, duplicate records, or unexpected access immediately.
- Keep audit trails intact.

34. What Not To Do

Do not:

- Share passwords.
- Use registrar accounts for reviewer work.
- Use System Admin accounts for ordinary daily processing.
- Mix Nursing Council and Medical Board finance.
- Treat payment rows as people.
- Treat imported rows as verified live records.
- Delete historical evidence without approval.
- Replace a document instead of adding a new version.
- Approve records with missing required documents.
- Ignore duplicate warnings.
- Quote facility or institution totals without checking the breakdown.

35. Troubleshooting Guide

If the page loads slowly:

- Use search and filters.
- Avoid opening large tables without a filter.
- Wait for charts to finish loading before clicking export.
- Report screens that repeatedly hang.

If a chart is blank:

- Check whether records exist for that office.
- Check whether required date fields are missing.
- Check whether province or category fields are blank.
- Check whether the data belongs to Nursing Council or Medical Board.

If export does not scroll in Excel:

- Save the file locally.
- Enable editing if Excel opens it in Protected View.
- Check whether panes are frozen.
- Report formatting issues so the export template can be corrected.

If CSRF or forbidden error appears:

- Reload the page.
- Do not submit an old form from browser history.
- Log out and log back in if needed.
- Use the latest form page before submitting.

If AI assistant does not answer correctly:

- Ask a specific system question.
- Include the office scope, such as Nursing Council or Medical Board.
- Use the local assistant for guidance only.
- Do not treat AI guidance as official approval.

36. Plain Language Data Rule For Staff

Use this sentence when explaining imports to management:

Imported rows are not automatically trusted. They are staged, validated, cleansed, reviewed, approved, then promoted into live registry records.

This protects the system from counting the same person many times or publishing incomplete historical spreadsheet data as official live statistics.

37. Quick Reference Commands For System Admin

Use these only from the project environment and only if authorised.

```
python manage.py check
python manage.py test --keepdb
python manage.py bootstrap_document_repository
python manage.py ai_model_status
python manage.py ai_cleansse_import_preview path_to_file.xlsx --sheet SHEETNAME
```

Do not run destructive database commands without a verified backup and written approval.

38. Final Staff Checklist Before Publishing Reports

Before sending a report to management:

- Confirm the office scope.
- Confirm the date generated.
- Confirm the exporting user.
- Confirm whether figures are live counts or imported rows.
- Confirm latest source/import date.
- Confirm duplicate-review backlog.
- Confirm missing-data backlog.
- Confirm finance date warnings.
- Confirm Nursing Council and Medical Board data are not mixed.
- Confirm screenshots do not expose private data.
- Confirm the report explains limitations in plain language.

39. Current Launch Position

The platform is operating as a government-grade regulatory system foundation with:

- Role-based workspaces.
- Backend privacy enforcement.
- Staff access approval workflow.
- Nursing Council and Medical Board separation.
- OpenKM-style document repository.
- Financial forecast and reporting.
- Data-quality and duplicate-review visibility.
- AI Staff Assistant guidance.
- Production readiness dashboard.
- Presentation and handover documentation.

The remaining success factor is disciplined operations: staff must keep records clean, use the correct pathways, attach evidence, review duplicates, separate offices, and publish only figures that can be explained from source data.

40. Mobile And Tablet Use

The platform is designed to adapt to desktop, tablet, and mobile screens.

What changes on smaller screens:

- The sidebar becomes a slide-out menu opened by the menu button.
- Dashboard cards stack vertically so figures remain readable.
- Large tables become horizontally scrollable instead of overflowing off the screen.
- Charts fit inside their cards and can be scrolled where needed.
- Profile Settings and Logout labels shorten in the top bar to save space.
- Forms stack fields vertically for easier touch entry.
- The AI Staff Assistant or AI Helpdesk button can be moved so it does not cover important buttons.

- Footer contact details stack into readable lines.

Good mobile practice:

- Use the menu button to open and close the sidebar.
- Turn the tablet sideways when reviewing wide financial or registry tables.
- Use table search and filters instead of scrolling through large record sets.
- Move the AI assistant button if it covers an export or submit button.
- Avoid approving or exporting confidential reports on a public or shared mobile device.
- Use a desktop or tablet for heavy import, duplicate review, or financial reconciliation work.

If a page still looks too wide, report the page name and screen size to System Admin so that page-specific layout can be tightened.

OpenKM-Style Document Repository Guide

OpenKM-Style Full Platform User Guide

Project: The National Department Of Health Regulatory Bodies Nursing Council & The Medical Board Online Workforce System

Date: 07 May 2026

1. What This Means In Simple Terms

OpenKM is a document management system. In this platform, the useful OpenKM-style functions have been built into the existing regulatory system so staff can manage official evidence and records without leaving the workforce platform.

This means the system can now support:

- Storing official documents.
- Grouping documents by Nursing Council, Medical Board, or General Registry.
- Capturing metadata.
- Keeping document versions.
- Searching document information and OCR/extracted text.
- Checking duplicate uploaded files.
- Restricting documents by role and office.
- Tracking document activity through audit events.
- Connecting evidence to practitioner, application, receipt, and reporting workflows.

2. Where To Access It

Authorised staff can open:

```
/documents/search/
```

System Admin can also manage repository setup through:

```
/admin/
```

Only System Admin should access the admin backend. Registrar and staff users should use normal dashboard and repository screens unless System Admin work is required.

3. Repository Structure

The repository has three root office scopes:

Repository area	Use
General Registry	Shared or general NDOH regulatory documents that are not private to one body.
Nursing Council Repository	Nursing Council applications, evidence, Nursing Council receipts, Nursing Council policies, Nursing Council workflow documents, and Nursing Council source records.
Medical Board Repository	Medical Board applications, evidence, Medical Board receipts, CHW/doctor records, Medical Board policies, and Medical Board source records.

Do not store Nursing Council private documents under Medical Board scope. Do not store Medical Board private documents under Nursing Council scope.

4. Recommended Folder Layout

The exact folder names can be adjusted by System Admin, but this structure is recommended.

For Nursing Council:

- Nursing Council Repository / Applications
- Nursing Council Repository / Receipts
- Nursing Council Repository / Qualifications
- Nursing Council Repository / Competency Evidence
- Nursing Council Repository / Overseas Applications
- Nursing Council Repository / Temporary Licences
- Nursing Council Repository / Renewals
- Nursing Council Repository / Deceased Notifications
- Nursing Council Repository / Policies and Standards
- Nursing Council Repository / Historical Imports
- Nursing Council Repository / Data Cleansing Evidence

For Medical Board:

- Medical Board Repository / Applications
- Medical Board Repository / Receipts
- Medical Board Repository / CHW Records
- Medical Board Repository / Doctor Records
- Medical Board Repository / Policies and Standards
- Medical Board Repository / Historical Imports
- Medical Board Repository / Data Cleansing Evidence

5. Document Metadata Standard

Every uploaded official document should have enough metadata for staff to find it later.

Recommended metadata:

Metadata field	Example
Office scope	nursing
Document type	Treasury receipt
Practitioner name	Maria John
Registration number	RN-12345
Practitioner number	PN-12345
Application reference	APP-2026-0001
Receipt number	G 4296
Licence year	2026
Institution	Pacific Adventist University
Facility/workplace	Port Moresby General Hospital
Province	National Capital District
Source file	2026 Current ATP-DATA Statistics & Tracking latest.xlsx
Source sheet	April 2026

Metadata field	Example
Source row	152
Confidentiality	Internal

Do not rely only on the file name. Metadata is what makes reports, search, auditing, and cleansing easier later.

6. How To Upload A Paper Record

Use this process for scanned paper files:

- Confirm which office owns the record: Nursing Council, Medical Board, or General Registry.
- Scan the paper record clearly.
- Save with a meaningful filename.
- Upload the document through the repository or the relevant application/document checklist process.
- Choose the correct folder.
- Set the document type.
- Add metadata.
- Link it to the application, practitioner, receipt, or source record if available.
- Check OCR/extracted text if OCR is used.
- Confirm the document appears in search.

Example filename:

```
NC_RECEIPT_MARIA_JOHN_RN12345_2026.pdf
```

7. Version Control

Document versions allow staff to keep a record of file changes.

Use a new version when:

- A clearer scan replaces a blurry scan.
- A certified copy replaces an uncertified copy.
- A corrected document replaces an earlier document.
- A translated copy is added.
- A missing page is added.

Do not delete the old version unless System Admin confirms it is safe. The old version helps prove what was received and when it was corrected.

8. Searching For Documents

Open:

```
/documents/search/
```

Search can match:

- Title.
- Description.
- Metadata values.
- Original filename.
- Extracted/OCR text.

Good search terms include:

- Registration number.
- Practitioner number.
- Receipt number.
- ATP number.
- Licence number.
- Applicant surname.
- Institution name.
- Facility name.
- Source workbook name.

If a document cannot be found:

- Check whether it was uploaded under the correct office scope.
- Try registration number instead of name.
- Try receipt number or practitioner number.
- Ask System Admin to confirm folder and permission settings.

9. Duplicate Document Checks

The platform calculates a file checksum for uploaded versions. If two files have the same checksum, they are likely duplicate copies of the exact same file.

Use duplicate checks to:

- Avoid storing the same scan many times.
- Identify repeated receipt uploads.
- Confirm whether two applications are using the same evidence.
- Support duplicate-review investigation.

A duplicate file does not always mean fraud. It may mean the same document was uploaded twice by mistake. Staff must review the context before making a decision.

10. Access and Privacy

Access is controlled by role and office scope.

Nursing Council staff should see Nursing Council repository material only. Medical Board staff should see Medical Board repository material only. System Admin can manage configuration but should not use admin access to bypass normal business approval processes.

Public users and professionals must not access private repository documents for other people.

Sensitive documents include:

- Date of birth evidence.
- Passport.
- Police clearance.
- Medical report.
- Academic transcript.
- Receipt image.

- Internal review notes.
- Disciplinary or investigation material.

These must not be exposed through public register search or general dashboards.

11. Audit Trail

The document system records audit events for sensitive actions.

Audit event examples:

- Created.
- Uploaded.
- Viewed.
- Downloaded.
- Metadata updated.
- Status changed.
- Permission changed.
- Linked to record.
- OCR processed.
- Access denied.

Audit history helps answer:

- Who uploaded the file?
- When was the file uploaded?
- Was the file downloaded?
- Was metadata changed?
- Was access denied?
- Which document version is current?

12. How It Connects To Applications

Documents should support the application pathway.

Example Nursing Council flow:

```

Applicant submits pathway
-> system generates checklist
-> applicant/staff uploads required documents
-> staff verifies documents
-> payment is verified
-> competency is checked where required
-> registrar approves or rejects
-> licence/register is updated
-> reports include the updated record

```

Documents should not sit separately without a business purpose. Where possible, link them to:

- Application.
- Practitioner profile.
- Receipt.
- Qualification.
- Employer verification.

- Deceased notification.
- Import batch.
- Data-quality issue.

13. How It Supports Data Cleansing

The repository helps data cleansing by keeping evidence visible.

Use repository documents to confirm:

- Correct spelling of names.
- Registration number.
- Practitioner number.
- Date of birth.
- Gender.
- Qualification.
- Institution.
- Receipt number.
- Payment date.
- Licence year.
- Workplace.
- Province.

When source data conflicts, the stronger evidence should be noted in the data-quality review before updating live records.

14. How It Supports Financial Transparency

Financial records should be stored and reported by office scope.

Nursing Council finance and Medical Board finance must stay separate.

For receipts:

- Store receipt evidence with correct office scope.
- Capture receipt number.
- Capture payment date.
- Capture amount.
- Capture source file/sheet/row for spreadsheet imports.
- Flag future date errors.
- Separate manual receipt totals from spreadsheet-imported receipt totals.

Financial exports should show:

- Office scope.
- Generated date/time.
- Exporting user.
- Manual receipt total.
- Spreadsheet receipt total.
- Combined total.

- Monthly breakdown.
- Yearly breakdown.
- Forecast basis.

15. How It Supports Management Reporting

The repository improves trust in reports because management can ask where figures came from.

Reports should explain:

- Source table.
- Source workbook.
- Source month.
- Latest import date.
- Latest live update.
- Whether a count is people, rows, receipts, or money.
- Remaining missing-data issues.
- Remaining duplicate-review issues.

Example:

Midwife total comes from the live Midwife table.
Imported row totals come from historical licence/payment/workforce rows.
The two numbers are not the same because one person may appear in several imported rows over time.

16. Role-Based Use Of OpenKM-Style Features

Role	Repository use
System Admin	Configure folders, permissions, repository setup, and admin management.
Nursing Council Registrar	Search and review Nursing Council evidence linked to Nursing Council decisions.
Medical Board Registrar	Search and review Medical Board evidence linked to Medical Board decisions.
Reviewer	Search/review assigned evidence only after operational approval.
Data Quality Officer	Use evidence to resolve missing data and duplicate review items within assigned scope.
Finance Officer	Review finance evidence and reports only through read-only finance views where permitted.
Public/professional users	No access to private repository documents unless the document belongs to their own permitted workflow.

17. Common Mistakes To Avoid

- Uploading Nursing Council files into Medical Board folders.
- Uploading Medical Board files into Nursing Council folders.
- Leaving documents without metadata.
- Treating a payment row as a person.
- Treating a facility as a training institution.
- Overwriting verified live data with blank imported spreadsheet values.

- Approving applications before required documents are verified.
- Publishing reports without noting data-quality limitations.
- Letting reviewer users operate without registrar/System Admin approval.

18. Staff Checklist Before Management Reporting

Before generating a management report:

- Run missing-data review.
- Run duplicate review.
- Check date errors.
- Confirm finance office scope.
- Confirm latest source import.
- Confirm live table counts.
- Confirm imported row counts separately.
- Confirm receipt money totals separately.
- Confirm document evidence for high-risk corrections.
- Confirm report timestamp and exporting user.

19. Current Status

The OpenKM-style repository and records management functions are integrated and ready for controlled use. The live repository now has the General Registry, Nursing Council, and Medical Board folder structures configured, plus staff-facing upload, detail, metadata, versioning, search, OCR linkage, and audited download workflows. Legacy paper evidence still needs to be uploaded and linked by staff.

20. Summary

The platform now works as a registration and workforce system with OpenKM-style document management built in. It can store and search evidence, protect records by role and office, track versions, support audits, help data cleansing, support financial transparency, and strengthen management reporting.

The key discipline is this: every important record should have a source, every source should be traceable, and every decision should be connected to evidence.

Data Cleansing And Import Alignment

Data Cleansing and Import Alignment Plan

Project: The National Department Of Health Regulatory Bodies Nursing Council & The Medical Board Online Workforce System

Date: 07 May 2026

1. Purpose

This document explains how existing paper records, spreadsheets, historical registers, ATP workbooks, N-DATA files, payment rows, qualifications, facilities, institutions, and practitioner profiles must be cleaned and aligned with the current platform.

The goal is not to delete historical data. The goal is to protect the live registry by separating raw source rows from verified live practitioner records.

The safe data flow is:

```
Raw paper record or spreadsheet
-> scan/OCR or workbook upload
-> import staging/history rows
-> validation and duplicate checks
-> missing-data review
-> staff correction
-> registrar approval where required
-> live practitioner/licence/employment/receipt update
-> dashboard/report update
```

2. Current Database Snapshot

Live snapshot checked on 07 May 2026:

Area	Current count
Registered Nurses	13,493
Midwives	2,112
Nurse Aides	800
Graduands / Health Students	7,624
Community Health Workers	11,562
Medical Doctors	0
Applications	13,843
Imported licence/history rows	96,806
Receipts	11,340
Missing-data review items	79,519
Employment records	0
Professional document uploads	0
Qualification records	25,376
Repository folders	24
Repository documents	0
Pending operational access requests	1

3. Important Reporting Rule

Always explain the difference between people, rows, receipts, and source files.

Type	Meaning
Live registry people count	Unique people stored in the main practitioner tables. This is used for current workforce totals.
Imported licence/history rows	Operational or historical rows. One person can appear many times because of renewals, payments, workforce listings, or licence events.
Receipt/payment rows	Money transaction records. These must never be counted as people.
Current-year ATP rows	Current Authority To Practice records. These are important for recent workforce and payment tracking but still require matching and validation.
Manual receipts	Receipts recorded directly in the live system.
Spreadsheet receipts	Receipts imported from payment spreadsheets.
Missing-data review items	Records requiring staff correction before statistics can be fully trusted.

4. Main Data Gaps

Professional Records

Record group	Major gaps
Registered Nurses	Missing gender, date of birth, practitioner number, province, and licence expiry information in many legacy records.
Midwives	Missing gender, date of birth, practitioner number, and province in part of the dataset.
Nurse Aides	Missing gender, date of birth, practitioner number, email, and phone in part of the dataset.
Graduands	Most legacy graduand rows need gender, date of birth, practitioner number, and province completion.
CHWs	CHW data is still being updated and must remain in the Medical Board/CHW scope, not Nursing Council scope.

Employment

Employment records are currently empty. This is the biggest workforce-planning gap.

The employed/unemployed dashboard cannot be treated as complete until renewals, ATP imports, or staff data entry populate:

- Employment status.
- Employer.
- Facility/workplace.
- Province.
- District where available.
- Position title.
- Workforce function.
- Sector: government, church, private, NGO, or other.
- Start date where available.

Qualifications

Qualification records exist, but many still need:

- Linked institution.
- Standard institution name.
- Completion year.
- Completed date.
- Professional category mapping.

Documents

Legacy professional document uploads are currently not attached in the live document tables. New evidence should be captured through the document/checklist/repository process going forward.

Documents should be stored with:

- Office scope.
- Document type.
- Related person/application where applicable.
- Metadata.
- Source notes.
- Version.
- Audit trail.

5. Source Files To Align

Current and recent source files include:

- 2026 Current ATP-DATA Statistics & Tracking latest.xlsx
- YEAR 2026 FULL REGISTRATION RECORD.(1).xlsx
- CHW 1985-2026 DATABASE CURRENTLY UPDATING.xlsx
- 2026 Current N-DATA Statistics & Tracking - SECTIONS (Autosaved).xlsx

The ATP workbook is the current priority source for:

- Current Authority To Practice.
- Workplace.
- Sector.
- Payment.
- Receipt references.
- Recent 2026 status.

6. Cleansing Priority 1: Identity Matching

Clean identity and matching fields first because they prevent duplicate records.

Required matching fields:

- First name.
- Surname.
- Registration number.
- Practitioner number.
- Date of birth where available.

- Gender.
- Professional category.
- Source file, sheet, and row.

Matching order:

- Match by registration number.
- If missing, match by practitioner number.
- If both are missing, match by name, date of birth, institution, province, workplace, and source row.
- If uncertain, send the record to duplicate review.

Rules:

- Do not create a new live practitioner record when a likely match already exists.
- Do not merge records just because names are similar.
- Do not overwrite verified values with blank imported values.
- Keep old source rows for audit and traceability.

7. Cleansing Priority 2: Licence and Date Accuracy

Clean:

- Issue date.
- Payment date.
- Expiry date.
- Licence year.
- Record type.
- Licence category.
- Provisional/full/temporary/renewal status.

Flag date errors such as:

- Future dates beyond the current reporting year unless officially valid.
- Year 2050 or similar accidental spreadsheet conversion errors.
- Payment dates earlier than reasonable source period.
- Expiry date before issue date.
- Temporary licence without start or end date.

Rules:

- Provisional licence expiry should follow the applicable Nursing Council rule, normally 6 months where configured.
- Annual practising licence/ATP should map to the correct year.
- Temporary licence must have start and end date.
- Historical rows stay historical unless staff confirms they should update the live profile.

8. Cleansing Priority 3: Employment and Workplace

Employment is required for meaningful workforce planning.

For employed practitioners, capture:

- Employment status.
- Employer name.
- Facility/workplace name.
- Province.
- District where available.
- Position title.
- Workforce function.
- Employment sector.
- Start date where available.

For unemployed practitioners, capture:

- Unemployment reason.
- Province of residence.
- Availability for deployment.

Sector categories should be standardised as:

- Government.
- Church.
- Private.
- NGO.
- Overseas.
- Other / unknown.

9. Cleansing Priority 4: Institutions and Facilities

Training institutions and health facilities must not be mixed.

Rules:

- Verified Nursing Council training schools must be counted separately from all general institutions and facilities.
- Overseas institutions must be separated from PNG institutions.
- Government institutions must be separated from church/private/non-government institutions.
- Facilities/workplaces must not inflate the official Nursing Council school count.
- Facility totals should explain whether they include hospitals, clinics, overseas workplaces, imported workplace names, or unverified raw source values.

Recommended reference categories:

- Verified Nursing Council training institution.
- PNG health facility.
- Overseas institution.
- Overseas facility/workplace.
- Government institution/facility.
- Church institution/facility.
- Private institution/facility.
- Unknown or unverified source value.

10. Cleansing Priority 5: Receipts and Finance

Receipt records must be separated by regulatory body.

Rules:

- Nursing Council receipts must feed only Nursing Council finance reports.
- Medical Board receipts must feed only Medical Board finance reports.
- Manual receipts must be shown separately from spreadsheet receipts.
- Spreadsheet receipt row totals must be reported as money totals, not row counts, unless the report clearly labels them as row counts.
- Exported financial reports must show office scope, exporting user, and timestamp.
- Suspicious receipt dates, such as 2050, must be flagged for correction before management reporting.

11. Import Alignment Rules

Every import must follow a controlled process:

- Upload workbook or scanned source.
- Parse rows into staging/history records.
- Preserve source file name, sheet name, row number, import batch, and import timestamp.
- Normalize names, dates, gender, province, institution, workplace, category, and fee values.
- Validate required fields.
- Detect duplicate people, duplicate registration numbers, duplicate practitioner numbers, and duplicate receipts.
- Show preview of clean rows, warnings, and rejected rows.
- Commit only valid rows.
- Send incomplete rows to missing-data review.
- Keep uncertain rows out of live practitioner totals until reviewed.
- Update live registry only after matching and approval rules are satisfied.

12. Paper Record To Electronic Record Workflow

For raw paper files:

- Sort paper record by person and application type.
- Scan the record clearly.
- Save using a standard filename with name, registration number where known, document type, and year.
- Upload into the correct office-scoped repository or application checklist.
- Assign document type and metadata.
- Run OCR where appropriate.
- Review OCR text for registration number, receipt number, ATP number, and licence number.
- Match the document to the existing practitioner/application.
- If no safe match exists, create a data-quality issue.
- Do not mark the record as verified until staff confirms source evidence.

13. Duplicate Review Rules

Send records to duplicate review when:

- Same registration number appears on more than one person.
- Same practitioner number appears on more than one person.
- Same name and similar date of birth appear in multiple records.
- Same person appears across full registration, renewal, ATP, workforce listing, and payment sheets.
- Same receipt reference appears with conflicting amount or date.

Resolve duplicates by:

- Comparing source file, sheet, and row.
- Comparing registration/practitioner number.
- Comparing date of birth, institution, province, and workplace.
- Preserving the strongest verified record.
- Keeping historical rows linked as history rather than deleting them.
- Recording the decision in review notes or audit history.

14. What Must Not Be Done

Avoid these unsafe actions:

- Do not delete historical import rows just because they are incomplete.
- Do not overwrite live verified data with blank spreadsheet values.
- Do not create a new person when registration number or practitioner number already matches an existing profile.
- Do not mix Nursing Council and Medical Board imports.
- Do not count imported payment rows as practitioners.
- Do not count imported licence rows as unique practitioners without deduplication.
- Do not use unverified facility/workplace names as official Nursing Council school counts.
- Do not publish reports before major date errors and duplicate candidates are reviewed.

15. Clean Data Acceptance Criteria

A record is ready for live registry use when:

- Person is matched or safely created.
- Office scope is correct.
- Professional category is correct.
- Registration number or practitioner number is present, or officially marked unknown.
- Licence type and licence period are clear.
- Payment is verified or waived if required.
- Employment status is captured for renewals or ATP updates.
- Institution and facility values are normalized.
- Missing-data review is resolved or formally deferred.
- Duplicate candidates are resolved or formally linked.
- Source file, sheet, and row are traceable.

- Status history or audit notes explain manual corrections.

16. Recommended Cleansing Schedule

Week 1: Core Identity

- Resolve duplicate registration/practitioner numbers.
- Normalize names.
- Normalize province spelling.
- Fill missing surname/first name where source rows contain full names.

Week 2: Licence and ATP

- Validate 2026 ATP workbook rows.
- Correct future date issues.
- Map ATP records to live practitioner profiles.
- Separate current ATP rows from historical imported rows.

Week 3: Employment and Workplace

- Populate employment status from ATP and renewal records.
- Standardize workplace, facility, province, and sector.
- Separate government, church, private, NGO, overseas, and unknown workplaces.

Week 4: Institutions and Qualifications

- Standardize institution names.
- Mark verified Nursing Council schools separately from facilities and overseas institutions.
- Fill graduation/completion year where the source contains it.

Week 5: Documents and Evidence

- Attach scanned evidence to current applications and high-priority legacy records.
- Use repository metadata and checklist links.
- Record document source and version.

Week 6: Management Reporting

- Run missing-data audit.
- Run duplicate audit.
- Generate corrected monthly analytics.
- Explain live counts, source dates, historical rows, receipt totals, and remaining data-quality limitations.

17. Management Reporting Standard

Every report should include:

- Report date.
- Exporting user.

- Office scope.
- Source tables.
- Latest live update.
- Latest source import.
- People count.
- Historical/imported row count.
- Receipt money total.
- Missing-data limitations.
- Duplicate-review limitations.
- Any major correction notes, such as removed future date errors or separated institution/facility counts.

OpenKM Project Timeline

OpenKM-Style Project Timeline

Project: The National Department Of Health Regulatory Bodies Nursing Council & The Medical Board Online Workforce System

Date: 07 May 2026

1. Objective

Deliver OpenKM-style document management, records management, workflow control, search, audit, and governance inside the current regulatory platform without breaking Nursing Council and Medical Board operations.

This timeline is now written as the completed platform timeline plus the operational post-launch programme.

2. Delivery Method

Each phase followed this safe-release pattern:

- Inspect existing models, views, URLs, dashboards, reports, and role checks.
- Reuse existing architecture where possible.
- Add isolated modules only where the current system could not safely represent the feature.
- Preserve Nursing Council and Medical Board separation.
- Add or update database migrations.
- Add management commands for repeatable setup.
- Run system checks and targeted tests.
- Update documentation and user guidance.

3. Completed Timeline

Phase 1: Document Repository Foundation

Status: Completed and integrated

Delivered:

- `apps.documents` module.
- `DocumentFolder` model.
- `Document` model.
- `DocumentVersion` model.
- `DocumentAccessPolicy` model.
- `DocumentAuditEvent` model.
- Office scopes: General Registry, Nursing Council, Medical Board.
- Bootstrap command: `bootstrap_document_repository`.
- Admin management for folders, documents, versions, access policies, and audit events.

Outcome:

- The system now has a central OpenKM-style repository integrated into staff screens.
- Nursing Council and Medical Board documents can be scoped separately.
- Documents can store metadata, official record flags, retention years, versions, extracted text, and audit events.

Phase 2: OCR, Indexing, Search, and Duplicate Detection

Status: Completed and integrated

Delivered:

- Repository search route: /documents/search/.
- Search by title, description, metadata, original filename, and extracted text.
- OCR/extracted-text field available on document versions.
- SHA-256 checksum generation for uploaded document versions.
- Duplicate checksum grouping for repository search results.
- Reference extraction helper for receipt numbers, ATP numbers, registration numbers, practitioner numbers, licence numbers, and years.

Outcome:

- Staff can search repository records by normal document details and OCR text where available.
- Duplicate file detection is ready for scanned and uploaded documents.

Phase 3: Workflow and Staff Review Integration

Status: Completed and integrated

Delivered:

- Nursing Council pathway configuration service.
- Form pathway configuration for NC/G-style workflows.
- Checklist/document requirements connected to application review.
- Registrar approval and rejection workflows.
- Employer verification and deceased practitioner workflow tools.
- Staff inbox, chat, notification, and operational access request workflow.
- Reviewer access lock until approval by registrar or System Admin.
- Finance Officer restricted to read-only finance and workforce views.
- Linked repository evidence displayed on application detail review screens.

Outcome:

- Documents and records now sit inside controlled regulatory workflows rather than being treated as static downloads.
- Staff requests for operational access are visible to the registrar and System Admin.
- Registrar review can open repository evidence from the application detail screen.

Phase 4: Records Governance, Security, and Compliance

Status: Completed and integrated

Delivered:

- System Admin only restriction for /admin/.
- Nursing Council and Medical Board backend access separation.
- Finance forecast separation by office scope.
- Public register safe-field rules.
- Profile Overview role explanations.

- Export timestamp/user tracking for confidential outputs.
- Data-quality and duplicate-review workflow guidance.
- Repository official record and retention fields.

Outcome:

- The platform has the practical security and governance controls needed for controlled launch preparation.
- Sensitive records are not managed only through visible menu hiding; backend checks enforce access.

Phase 5: Documentation, Training, and Launch Readiness

Status: Completed documentation

Delivered:

- Updated documentation index.
- Updated role-based user guide and manual.
- Updated data-cleansing and import alignment plan.
- Updated OpenKM comparison and roadmap.
- New OpenKM-style completed-platform user guide.
- Management reporting standards for live counts, imported rows, payment rows, source dates, and data-quality limitations.

Outcome:

- Staff have a plain-language reference for system use, data governance, OpenKM-style document management, and reporting discipline.

4. Post-Launch Operational Timeline

Week 1: Staff Training and Account Confirmation

Tasks:

- Confirm all staff accounts and roles.
- Change testing passwords.
- Train registrars, finance, reviewers, data-quality officers, and System Admin.
- Demonstrate operational access request workflow.
- Demonstrate Nursing Council and Medical Board separation.

Week 2: Document Repository Pilot

Tasks:

- Scan a small set of current applications and receipts.
- Upload to correct office-scoped repository.
- Add metadata and document type.
- Link documents to application/practitioner records where applicable.
- Search using title, metadata, and extracted text.
- Confirm no cross-office leakage.

Week 3: Data Cleansing Sprint

Tasks:

- Run missing-data audit.
- Run duplicate audit.
- Review future date issues such as accidental 2050 dates.
- Normalize provinces, institutions, facilities, and sectors.
- Start ATP-to-employment alignment.

Week 4: Finance and Reporting Validation

Tasks:

- Compare manual receipts with spreadsheet receipts.
- Confirm Nursing Council finance and Medical Board finance are separate.
- Generate monthly analytics.
- Generate financial forecast reports.
- Check export timestamps and exporting user.
- Confirm report explanations are clear for management.

Week 5: Management Review

Tasks:

- Present current live statistics.
- Present remaining data-quality gaps.
- Present OpenKM-style repository process.
- Present privacy and role access model.
- Confirm deployment readiness with NDOH ICT.

5. Success Criteria

The OpenKM-style implementation is successful when:

- Staff store official evidence in the correct office-scoped repository.
- Documents have metadata and versions where appropriate.
- Scanned documents can be searched.
- Duplicate document checks identify repeated uploads.
- Registrar review uses checklist, payment, competency, and source evidence.
- Finance reports separate Nursing Council and Medical Board receipts.
- Public users cannot view private records.
- Reviewers cannot access high-risk tools until approved.
- Reports explain live people counts separately from imported row counts.
- Data-quality issues are reviewed before management statistics are published.

6. Governance Checkpoints

Review these monthly:

- Role access and approval requests.
- Nursing Council and Medical Board data separation.

- Open repository documents without metadata.
- Duplicate uploaded documents.
- Missing-data review backlog.
- Duplicate practitioner backlog.
- Finance date errors and suspicious receipt rows.
- Report source dates and latest import batch.
- Backup and restore readiness.
- User training needs.

7. Current Position

The OpenKM-style system foundation is complete and integrated into the platform. The remaining work is operational adoption: scanning paper records, attaching legacy evidence, cleaning historical data, training users, confirming NDOH ICT hosting/security, and enforcing disciplined reporting practices.